# MEFENAMIC ACID

Max 7 days use

Take with food

**CHEMICAL NAME** 2-[(2,3-dimethylphenyl)amino]benzoic acid
**CLASS** Non-selective NSAID (fenamate / anthranilic acid derivative)
**TRADE NAMES** Ponstan · Ponstel · Meftal

## THERAPEUTIC USES / INDICATIONS

- Primary dysmenorrhoea.
- Menorrhagia (heavy menstrual bleeding).
- Mild to moderate pain: dental, musculoskeletal, headache.

## PHARMACODYNAMICS

- Inhibits **COX** enzymes and may block prostaglandin receptors.
- Reduces uterine prostaglandins → less cramping and menstrual blood loss.

## ADMINISTRATION

- **PO** capsules, tablets or suspension, with food.
- Usually limited to 7 days of treatment.

## SIDE & ADVERSE EFFECTS

### SIDE EFFECTS (COMMON)

- Diarrhoea (stop if it develops), nausea, dyspepsia.
- Drowsiness, dizziness, headache.
- Rash.

### ADVERSE EFFECTS (SERIOUS)

- GI bleeding and ulceration.
- Haemolytic anaemia; thrombocytopenia.
- Renal impairment (non-oliguric failure in dehydration).
- Seizures in overdose.

## DOSAGES

- **Adult:** 500 mg three times daily.
- **Dysmenorrhoea/menorrhagia:** start at onset of menses or pain; continue for 3–5 days.
- **Children >6 months:** 25 mg/kg/day in divided doses (50 mg/5 mL suspension), up to three times daily; short courses.

## CONTRAINDICATIONS & CAUTIONS

- Active GI ulcer or inflammatory bowel disease.
- Renal or hepatic impairment.
- NSAID-sensitive asthma.
- Third trimester of pregnancy.

### KEY INTERACTIONS

- **Anticoagulants:** ↑ bleeding risk.
- **Lithium, methotrexate:** ↑ toxicity.
- **ACE inhibitors, diuretics:** ↑ renal risk.

## PHARMACOKINETICS

- **Onset:** 1 h. **Peak:** 2–4 h. **Duration:** 6 h.
- Metabolised in liver (CYP2C9).
- Excreted in urine and faeces. **t½:** about 2 h.

## NURSING MANAGEMENT

- Assess pain and menstrual pattern.
- Give with food.
- Stop drug and report if diarrhoea or rash develops.
- Monitor CBC with prolonged or repeated courses.
- Watch for GI bleeding.

## PATIENT EDUCATION

- Take with food; do not use longer than 7 days.
- Stop and report diarrhoea, rash or dark stools.
- Start at the beginning of the period for best effect.
- Avoid other NSAIDs.

## KEY POINTS

- Fenamate NSAID; main use dysmenorrhoea and menorrhagia.
- Short courses only (≤7 days).
- Stop if diarrhoea occurs.
- 500 mg TDS with food.

**SOURCES CHECKED** **[12]** Mefenamic Acid 250 mg Capsules (UK SmPC) **[13]** Mefenamic Acid 50 mg/5 ml Suspension (UK SmPC) **[14]** Mefenamic Acid, oral (US label) **[15]** Mefenamic acid, oral (US label)
*Full references p. 432–457*